THE MATERNITY METHOD

NCLEX CHEAT SHEET

Ectopic & Molar Pregnancy

Maternal & Newborn › Bleeding in Early Pregnancy

The Big Picture

Ectopic pregnancy = the embryo implants **outside the uterus**, usually in a **fallopian tube**. It can **rupture** and cause life-threatening bleeding. **Molar pregnancy** (hydatidiform mole) = **abnormal placental tissue** grows into grape-like cysts instead of a normal pregnancy. It makes **very high hCG** and needs close follow-up because it can turn into cancer.

KNOW THE NUMBERS

~95% of ectopics are in a **tube**	**48 hrs** Early hCG should about **double**	**Shoulder pain** = blood under the diaphragm: **rupture**	**< 20 wks** Preeclampsia this early? Think **molar**	**0** hCG must return to **zero** after either one

Ectopic vs. Molar at a Glance

Ectopic Pregnancy

- **Missed period** + **one-sided** lower belly pain + spotting
- **Rupture:** sudden **sharp, severe** pain, **shoulder pain**, dizziness, **shock** signs
- **Ultrasound:** no pregnancy in the uterus
- **hCG** rises **more slowly** than expected

Molar Pregnancy

- **Uterus bigger than dates**
- **Very high hCG → severe vomiting**
- Dark brown ("prune juice") **bleeding**; may pass **grape-like** clusters
- **No fetal heart tones**; early **preeclampsia**; hyperthyroid signs
- **Ultrasound:** "snowstorm" pattern

Ectopic: Who's at Risk

- **Previous ectopic**
- **PID** or past chlamydia/gonorrhea (scarred tubes)
- **Tubal surgery** or tubal ligation
- Pregnancy with an **IUD** in place
- **IVF**, smoking

Ectopic: Treatment

- **Unruptured + stable + small:** methotrexate IM (stops cell growth)
- **Surgery:** laparoscopy to open or remove the tube
- **Ruptured = emergency:** 2 large-bore IVs, fluids, type & crossmatch, prepare for **surgery**
- **RhoGAM** if mom is Rh−

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MEMORY HACK · METHOTREXATE TEACHING

On methotrexate? Skip the FANS

F	A	N	S
Folic acid & prenatal vitamins (they block the drug)	**Alcohol**	**NSAIDs** (use acetaminophen)	**Sun** exposure and **sex** until hCG is zero

Molar Pregnancy: Nursing Care

- **Suction D&C** to remove the tissue; send it to pathology
- **RhoGAM** if mom is Rh−
- **Follow hCG until it's normal**, then keep checking for months as ordered. A rise means **leftover tissue or cancer** (choriocarcinoma).
- **Reliable birth control** during follow-up. A new pregnancy would raise hCG and **hide** a problem.
- Grief support: this is still a **pregnancy loss**

RED FLAGS · STOP AND CALL THE PROVIDER

Sudden severe belly pain · **Shoulder pain** · **Dizziness, fainting, low BP, fast HR** · **Heavy bleeding** · hCG that **plateaus or rises** after treatment

MUST-KNOW FOR THE NCLEX

- **One-sided pain + missed period + spotting** = rule out ectopic.
- **Shoulder pain** with ectopic = rupture → treat for **shock** and prepare for surgery.
- **Methotrexate:** no folic acid, alcohol, NSAIDs, sun, or sex until hCG is zero.
- **Molar:** big uterus, very high hCG, prune-juice bleeding. Avoid pregnancy during hCG follow-up.

QUICK CHECK · TEST YOURSELF

1. Most ectopic pregnancies implant in the _____.
2. Shoulder pain in ectopic pregnancy means _____.
3. A patient on methotrexate should stop taking _____ _____.
4. A molar pregnancy ultrasound shows a _____ pattern.
5. After a molar pregnancy, why avoid getting pregnant during follow-up?

Answers: 1. fallopian tube · 2. rupture (blood irritating the diaphragm) · 3. folic acid (prenatal vitamins) · 4. snowstorm · 5. Rising hCG would hide leftover tissue or cancer

For study use only. Always follow facility protocols, current guidelines, and provider orders.